Section J: Client Records

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Effective date: Jan. 1, 2027. Review cycle: once a year.

Regulatory basis: K.A.R. 28-51-110(a), (c), (d), (e); K.A.R. 28-51-104; K.A.R. 28-51-117(e); KDHE Application Guide, Client Records section.

J-1. Client Records Policy

Golden Rule Home Care & Concierge Services LLC keeps a client record for every client who receives supportive care services.[1] Each record holds pertinent past and current findings and is kept in line with accepted professional standards.[2] The administrator is responsible for opening, maintaining, and closing each record.[3] Each client record is kept both on paper and as an electronic copy.[4]

J-2. Contents of the Client Record

Each client record contains at least the following:[5]

1. The plan of care;[6]
2. The name of the client's physician, nurse practitioner, clinical nurse specialist, or physician assistant;[7]
3. Physician orders for drugs, diet, treatment, and activity;[8]
4. Signed and dated admission notes;[9]
5. Documentation of supportive care services provided, the date and time the worker checked in and out, and a confirmation that the services were provided;[10]
6. A copy of progress notes;[11]
7. The date of each on-site supervision visit required by K.A.R. 28-51-117; and[12]
8. The discharge summary report.[13]

The record also holds the client assessment, the signed client bill of rights acknowledgment from Section K, and the written assignments for each worker from Section G.[14]

J-3. Client Record Retention

Each client record is kept in a retrievable form for at least five years after the date of the client's last discharge.[15] If the agency stops operating, Golden Rule Home Care & Concierge Services LLC keeps the records for the rest of the five years as their custodian, with Taylor Anderson as alternate custodian.[16] At least 30 days before closing, the LLC notifies KDHE in writing of the reason for closing, the closing date, where active and inactive records will be held, and the custodian's name and address, as K.A.R. 28-51-101(k)(1) requires.[17]

J-4. Safeguarding Client Records

Client records are protected against loss and against unauthorized review or use.[18] Paper records are kept in a locked file cabinet in the office at . Electronic copies are kept on a locked, password-protected computer that only the administrator uses.[19] Only the administrator and staff assigned to the client may see a record, and only for the client's care.[20] Records are never left in a vehicle or open in a client's home.[19]

J-5. Use, Removal, and Release of Information

Original records stay in the office.[21] A worker may take only the written assignment and visit notes needed for a shift.[21] The worker returns paper visit notes to the office by the end of the same day, and the administrator enters them on the computer described in J-4.[21] Workers never use that computer.[19] Copies leave the office only with the administrator's approval.[21]

The client's or guardian's written consent shall be required for the release of information if that release is not required by law.[22] Each release is logged with the date, what was released, to whom, and the consent on file.[23] Requests from KDHE or other agencies with legal authority are handled by the administrator.[22][24]

J-6. Admission Notes

After the client assessment, the manager completes, signs, and dates an admission note.[25] The note specifies the relevant diagnoses; the client's health history; environmental, safety, and social factors of the client's home; the client's nutritional requirements, medications, and treatments; the client's functional status and abilities; and the client's physical and mental levels of functioning.[26] It also records the date of admission, the services agreed on, and the client's receipt of the bill of rights.[27]

J-7. Documentation of Supportive Care Services

At every visit the worker records the date, time in, time out, each service provided, and confirmation that the services were given.[28] The worker also notes any change in the client's condition and reports it to the manager as the written assignment directs.[29] Entries are made the same day.[30]

J-8. Progress Notes and Charting Tiers

A progress note is a dated, written note about a visit.[31] Each progress note gives the name of the worker, the date and time of the visit, the services the client received, how the client responded, and the date of the next visit and the services planned for it.[32] It shows the client's functional status and abilities compared with earlier visits, and the client signs it to confirm the services were received.[33] A copy of every progress note is kept in the client record.[11]

How much is charted at each visit depends on the client's tier.[34] Every client is placed in one of two tiers, high risk or low risk, based on the qualifiers below.[34] Records for both tiers are kept on paper and on the computer described in J-4.[4]

High-risk tier

A high-risk client is one whose health or safety is changing, or could change quickly, so that close tracking from visit to visit is needed.[35] A client is placed in the high-risk tier if any one of these applies:[35]

1. The client has fallen within the last **[TAYLOR TO DECIDE: number of days, such as 90]**.[35]
2. The client has been in the hospital or emergency room, or has had surgery, within the last **[TAYLOR TO DECIDE: number of days, such as 30]**.[35]
3. The client's health is in decline, or the client's condition, diagnosis, or medications have changed recently.[35]
4. The client has memory loss or confusion that affects safety at home.[35]
5. The client needs help walking, moving from bed or chair, bathing, or with other personal care.[35]
6. The client's physician, family, or responsible party has asked for an update after each visit.[35]
7. A report of suspected abuse, neglect, or exploitation involving the client is open under Section F.[35]

[TAYLOR TO DECIDE: falls, hospital stays, and poor or quickly changing health come from your examples. The emergency room and surgery in qualifier 2, the diagnosis and medication changes in qualifier 3, and qualifiers 4 through 7 are our suggestions. Keep, cut, or change any of them.]

A high-risk client gets a full progress note at every visit.[32] Notes may be shared with family members the client has approved in writing.[36]

Low-risk tier

A low-risk client is one whose health is stable and who receives mainly companionship, conversation, errands, or light help around the home.[35] A client is placed in the low-risk tier only if none of the high-risk qualifiers apply and all of these are true:[35]

1. The client's health and daily abilities have been stable for at least **[TAYLOR TO DECIDE: number of days, such as 30]**.[35]
2. The services are limited to companionship, conversation, errands, light housekeeping, or other services that do not involve hands-on personal care.[35]
3. No one has asked for an update after each visit.[35]

[TAYLOR TO DECIDE: companionship and conversation come from your examples. Errands, light housekeeping, and the other services listed are our suggestions. Keep, cut, or change them.]

A low-risk client gets a short progress note at every visit that still covers the required items above, plus a full progress note at least **[TAYLOR TO DECIDE: how often, such as once a month]** and any time a high-risk qualifier appears.[37] **[VERIFY: confirm with KDHE that a short-form note meets the progress note definition in K.A.R. 28-51-100 for stable clients.]** The steps for writing each kind of note are in the training manual.[35]

Assigning and reviewing the tier

The administrator assigns the tier at admission and records it in the plan of care.[35] The tier is reviewed at every plan of care review, at least every 60 days, at every supervision visit under J-9, and any time the client's condition changes.[38] A low-risk client moves to the high-risk tier as soon as any high-risk qualifier applies,

and the worker tells the administrator the same day.[35] A high-risk client moves back to the low-risk tier only after the administrator confirms that no high-risk qualifier has applied for at least **[TAYLOR TO DECIDE: number of days, such as 30]**.[35] Each tier change is dated and noted in the client record.[35]

Whatever the tier, the visit record in J-7 is completed at every visit.[28]

J-9. Supervision Visits in the Record

The manager visits each client on site at least every three months to supervise the worker.[39] The client record shows the date of each visit, the client's satisfaction with services, and the worker's adherence to the plan of care.[40] Section I sets the full supervision policy.[41]

J-10. Discharge Summary

When services end, the administrator writes a discharge summary, signed by a manager, within five business days after discharge.[42] The summary gives the date and reason for discharge, the services provided, the client's treatment and response in line with the plan of care, the client's condition and final disposition at discharge, and any referrals made.[43] The summary is filed in the client record, and the five-year retention period starts on the date of last discharge.[44]

Adoption

The governing body of Golden Rule Home Care & Concierge Services LLC adopts the policies in this section.[45] Taylor Anderson, as sole member and owner of the LLC, approves them and reviews them once a year or sooner when a rule changes.[46]

Signature: _____ **Date:** _____

Taylor Anderson, Owner, Golden Rule Home Care & Concierge Services LLC

Effective date: Jan. 1, 2027. Next review date: Jan. 1, 2028.

Client Review

Client redlines: mark any changes on this section, then sign and date below. A digital signature (for example DocuSign) is accepted in place of ink.

Client signature: _____

Printed name: _____

Date: _____

Client signature (digital)

Date signed

Notes

1. K.A.R. 28-51-110(a); K.A.R. 28-51-104(d)(2)(A). Required: A client record with pertinent past and current findings is kept for each client in line with accepted professional standards. Source: Kansas Register, May 5, 2022.
2. K.A.R. 28-51-110(a). Required: The record holds pertinent past and current findings and is kept in line with accepted professional standards. Source: Kansas Register, May 5, 2022.
3. K.A.R. 28-51-103(b)(1)(E). Required: The administrator maintains appropriate administrative records. Naming the administrator responsible for client records is agency choice. Source: Kansas Register, May 5, 2022.
4. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-110(d), (e), retention and safeguards, Kansas Register.
5. K.A.R. 28-51-110(c). Required: Each client record contains at least the listed items. Source: Kansas Register, May 5, 2022.
6. K.A.R. 28-51-110(c)(1). Required: The plan of care. Source: Kansas Register, May 5, 2022.
7. K.A.R. 28-51-110(c)(2). Required: The name of the client's physician, nurse practitioner, clinical nurse specialist or physician assistant. Source: Kansas Register, May 5, 2022.
8. K.A.R. 28-51-110(c)(3). Required: Physician orders for drugs, diet, treatment and activity. Source: Kansas Register, May 5, 2022.
9. K.A.R. 28-51-110(c)(4). Required: Signed and dated admission notes. Source: Kansas Register, May 5, 2022.
10. K.A.R. 28-51-110(c)(5). Required: Documentation of services provided, check-in and check-out times, and confirmation the services were provided. Source: Kansas Register, May 5, 2022.
11. K.A.R. 28-51-110(c)(6). Required: A copy of progress notes. Source: Kansas Register, May 5, 2022.
12. K.A.R. 28-51-110(c)(7). Required: The date of each on-site supervision visit required by K.A.R. 28-51-117. Source: Kansas Register, May 5, 2022.
13. K.A.R. 28-51-110(c)(8). Required: The discharge summary report. Source: Kansas Register, May 5, 2022.
14. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-110(c), client record contents, Kansas Register.
15. K.A.R. 28-51-110(d). Required: Each client record is kept in retrievable form for at least five years after the client's last discharge. Source: Kansas Register, May 5, 2022.
16. K.A.R. 28-51-110(d); K.A.R. 28-51-101(k)(2). Required: If the licensee stops operating, it makes provision for keeping the records. No rule found limits who may serve as custodian. Source: Kansas Register, May 5, 2022.
17. K.A.R. 28-51-101(k)(1). Required: At least 30 days before permanent closure, the licensee notifies KDHE in writing of the reason, the closing date, the location of active and inactive records, and the name and address of the custodian. Source: Kansas Register, May 5, 2022.

18. K.A.R. 28-51-110(e). Required: Written policies cover use and removal of documents from the client record and the conditions for release of information. How the records are protected is agency choice. Source: Kansas Register, May 5, 2022.
19. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-110(e), safeguard against loss or unauthorized use, Kansas Register.
20. K.A.R. 28-51-111(a)(7). Required: Clients have the right to confidentiality of all client records, communications and personal information. Source: Kansas Register, May 5, 2022.
21. K.A.R. 28-51-110(e). Required: Written policies cover use and removal of documents from the client record. The specific limits are agency choice. Source: Kansas Register, May 5, 2022.
22. K.A.R. 28-51-110(e). Required: The client's or guardian's written consent is required for release of information not required by law. Source: Kansas Register, May 5, 2022.
23. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-110(e), conditions for release of information, Kansas Register.
24. K.A.R. 28-51-101(i). Required: The administrator or alternate administrator provides all client records and administrative records to the KDHE surveyor within 30 minutes of the surveyor's request. The secretary may extend the time. Source: Kansas Register, May 5, 2022.
25. K.A.R. 28-51-100(b)(1), (b)(2)(B); K.A.R. 28-51-110(c)(4). Required: A signed and dated admission note is completed by a manager after the initial assessment and is used to develop the plan of care. Source: Kansas Register, May 5, 2022.
26. K.A.R. 28-51-100(b)(1)(A) to (F). Required: The admission note specifies relevant diagnoses, health history, environmental, safety and social factors in the home, nutritional needs, medications and treatments, functional status and abilities, and physical and mental levels of functioning. Source: Kansas Register, May 5, 2022.
27. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-100(b)(1), admission note, Kansas Register.
28. K.A.R. 28-51-110(c)(5). Required: Documentation of services provided, the date and time the worker checked in and out, and confirmation the services were provided. Source: Kansas Register, May 5, 2022.
29. K.A.R. 28-51-117(f)(2)(B), (C). Required: Written assignments specify when the worker reports to the supervisor and require documenting services at each visit. Source: Kansas Register, May 5, 2022.
30. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-110(a), accepted professional standards, Kansas Register.
31. K.A.R. 28-51-100(rr)(1). Required: A progress note is a dated, written notation documenting a visit and summarizing treatment, response and functional status. Source: Kansas Register, May 5, 2022.
32. K.A.R. 28-51-100(rr)(1)(A) to (E). Required: Each progress note includes the name of the person who provided the treatment, the date and time, the treatment received, the client's response, and the date of the next visit and treatment planned. Source: Kansas Register, May 5, 2022.
33. K.A.R. 28-51-100(rr)(2). Required: Each progress note is clear and specific, shows functional status and abilities compared to other visits, and is signed by the client to confirm receipt. Source: Kansas Register, May 5, 2022.
34. K.A.R. 28-51-100(rr); K.A.R. 28-51-104(c)(3)(G). Allowed: Progress notes are kept in the client record, and every progress note must meet the full definition. No rule found sets charting tiers, so tiers are agency choice as long as each note meets the definition. Source: Kansas Register, May 5, 2022.
35. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-100(rr), progress note definition, Kansas Register.
36. K.A.R. 28-51-110(e). Required: The client's or guardian's written consent is required for release of information not required by law. Source: Kansas Register, May 5, 2022.
37. K.A.R. 28-51-100(rr)(1), (2). Required: Every progress note, short or full, must include the five listed items, show functional status compared with other visits, and be signed by the client. Source: Kansas Register, May 5, 2022.
38. K.A.R. 28-51-104(c)(3)(F). Required: The manager reviews the plan of care at least every 60 days. Reviewing the tier at the same time is agency choice. Source: Kansas Register, May 5, 2022.
39. K.A.R. 28-51-117(e)(2); K.A.R. 28-51-104(c)(3)(K). Required: On-site supervision of each worker at least every three months, including client satisfaction and the worker's adherence to the plan of care. Source: Kansas Register, May 5, 2022.
40. K.A.R. 28-51-110(c)(7); K.A.R. 28-51-117(e)(2). Required: The client record holds the date of each supervision visit, and the visit includes client satisfaction and adherence to the plan of care. Source: Kansas Register, May 5, 2022.
41. No state or federal rule applies; agency choice. Framing source: K.A.R. 28-51-117(e), supervision, Kansas Register.
42. K.A.R. 28-51-100(q)(2); K.A.R. 28-51-110(c)(8). Required: The client record holds a discharge summary report, a concise written statement signed by a manager reflecting the client's treatment and response under the plan of care and final disposition at discharge. The five-business-day deadline is agency choice. Source: Kansas Register, May 5, 2022.

43. K.A.R. 28-51-100(q)(2). Required: The summary reflects treatment and response under the plan of care and the client's final disposition at discharge. The other listed contents are agency choice. Source: Kansas Register, May 5, 2022.
44. K.A.R. 28-51-110(c)(8), (d). Required: The discharge summary is part of the client record, and retention runs at least five years from the last discharge. Source: Kansas Register, May 5, 2022.
45. K.A.R. 28-51-103(a)(3). Required: The governing body adopts, revises and approves the agency's policies and procedures as needed. Source: Kansas Register, May 5, 2022.
46. K.A.R. 28-51-103(a)(3). Allowed: The governing body revises policies as needed. A yearly review is agency choice. Source: Kansas Register, May 5, 2022.